

HOSPITAL COMPLIANCE & REGULATORY MANUAL

Administrator Reference Dataset

Version: 1.0

Document Owner: Quality & Compliance Department

Applicable To: Hospital Administrators, Compliance Officers, Quality Managers, Medical Superintendents

SECTION 1: NABH COMPLIANCE GUIDELINES SUMMARY

Purpose

To establish standards for patient safety, quality healthcare delivery, risk management, documentation, and operational excellence.

Key NABH Compliance Areas

1. Access, Assessment & Continuity of Care

Requirements:

- Defined admission process
- Patient assessment protocols
- Continuity of care documentation
- Referral management

Administrator Responsibilities:

- Monitor admission turnaround times
- Ensure assessment compliance
- Maintain referral records

2. Care of Patients

Requirements:

- Clinical care pathways
- Treatment documentation
- High-risk patient management

Administrator Controls:

- Clinical audit reviews
- Incident monitoring
- Outcome reporting

3. Medication Management

Requirements:

- Safe prescribing
- Medication reconciliation
- High-alert medication controls

Compliance Indicators:

- Medication error rate
- Adverse drug events
- Pharmacy audits

4. Patient Rights & Education

Patients have rights to:

- Information
- Privacy
- Confidentiality
- Informed consent
- Safe treatment
- Complaint resolution

Administrator Responsibilities:

- Display patient rights
- Monitor grievance handling
- Track consent compliance

5. Infection Control

Requirements:

- Hand hygiene programs
- Isolation procedures
- Biomedical waste management
- HAI surveillance

KPIs:

- CAUTI Rate
- CLABSI Rate
- SSI Rate

6. Information Management System

Requirements:

- Medical record security
- Record retention
- Access controls
- Audit trails

Administrator Responsibilities:

- EHR governance
- Data protection monitoring
- Backup verification

SECTION 2: HIPAA SUMMARY FOR HEALTHCARE ADMINISTRATORS

Purpose

Protect patient health information and regulate disclosure of healthcare data.

Protected Health Information (PHI)

Examples:

- Patient Name
- Medical Record Number

- Diagnosis
- Insurance Details
- Laboratory Reports
- Treatment Information

HIPAA Privacy Principles

Access only when:

- Required for treatment
- Required for operations
- Required for payment processing

Prohibited:

- Unauthorized sharing
- Social media disclosure
- Personal email transmission

Minimum Necessary Rule

Employees may access only:

- Information needed for assigned duties

Examples:

Billing Staff:

Access billing records only

Nursing Staff:

Access assigned patient records only

Breach Management

Potential Breach Examples:

- Lost laptop
- Misdirected email
- Unauthorized access
- Stolen patient records

Administrator Actions:

1. Investigate incident
2. Notify compliance officer
3. Document findings
4. Implement corrective actions

SECTION 3: DATA PRIVACY POLICY

Objective

Protect personal and health information collected by the hospital.

Data Categories

Patient Data

- Demographics
- Clinical records
- Diagnostic reports
- Insurance information

Employee Data

- Payroll records
- Credentials
- Performance records

Vendor Data

- Contracts
- Financial information

Data Collection Principles

The hospital shall collect only:

- Necessary information
- Lawfully obtained information
- Purpose-specific information

Data Storage Requirements

Electronic Records:

- Encryption mandatory
- Role-based access
- Audit logging enabled

Physical Records:

- Locked storage
- Controlled access

Data Sharing Rules

Allowed:

- Treating physicians
- Insurance partners
- Regulatory authorities

Prohibited:

- Unauthorized third parties
- Personal devices
- Unapproved cloud storage

Data Retention Schedule

Record Type	Retention
Medical Records	10 Years
Claims Records	10 Years
Billing Records	8 Years
HR Records	7 Years

Data Disposal

Electronic:

- Secure deletion

- Storage sanitization

Physical:

- Cross-cut shredding
- Authorized destruction

SECTION 4: PATIENT CONSENT POLICY

Purpose

Ensure patients provide informed authorization before treatment.

Consent Categories

General Consent

Obtained during admission.

Includes:

- Examination
- Routine treatment
- Nursing care

Informed Consent

Required for:

- Surgery
- Invasive procedures
- Anaesthesia
- Blood transfusion

Consent Must Include:

- Diagnosis
- Procedure details
- Benefits
- Risks

- Alternatives
- Consequences of refusal

High-Risk Consent

Required for:

- ICU admission
- Organ support
- Experimental therapy

Telemedicine Consent

Required before:

- Virtual consultation
- Remote monitoring

Research Consent

Required before:

- Clinical trials
- Research participation
- Data publication

Consent Validation Rules

Valid Consent Requires:

- Patient identification
- Capacity assessment
- Voluntary agreement
- Witness (where applicable)
- Documentation

Consent Withdrawal

Patients may withdraw consent unless:

- Emergency treatment required
- Legal obligations apply

SECTION 5: PATIENT RIGHTS POLICY

Rights of Patients

Patients have the right to:

- Respect and dignity
- Privacy
- Confidentiality
- Safe care
- Access to information
- Refuse treatment
- Obtain second opinion
- Complaint resolution

Grievance Management

Complaint Channels:

- Patient Relations Desk
- Email
- Hotline
- Written Complaint

Resolution Targets:

Level 1: 24 Hours

Level 2: 3 Days

Level 3: 7 Days

SECTION 6: MEDICAL RECORD MANAGEMENT POLICY

Objectives

Ensure accurate and secure healthcare records.

Medical Record Components

- Admission Notes
- Progress Notes
- Nursing Notes
- Laboratory Reports
- Imaging Reports
- Consent Forms
- Discharge Summary

Record Completion SLA

Discharge Summary:
Within 24 hours of discharge

Operative Notes:
Within 12 hours

Death Summary:
Within 24 hours

SECTION 7: INCIDENT REPORTING POLICY

Reportable Events

- Patient fall
- Medication error
- Wrong patient identification
- Data breach
- Equipment malfunction

Reporting Timeline

Critical Incident:
Immediate

Serious Incident:
Within 2 Hours

Minor Incident:
Within 24 Hours

Escalation Matrix

Level 1:
Department Manager

Level 2:
Quality Manager

Level 3:
Medical Superintendent

Level 4:
Hospital Administrator

Level 5:
Chief Executive Officer

SECTION 8: AUDIT & COMPLIANCE MONITORING

Internal Audits

Monthly:

- Documentation Audit
- Consent Audit
- Infection Control Audit

Quarterly:

- Regulatory Compliance Audit
- Data Privacy Audit

Annual:

- NABH Readiness Assessment

Compliance KPIs

KPI	Target
Consent Compliance	> 98%
Documentation Completion	> 99%
Privacy Breaches	0
Incident Reporting Compliance	100%
Audit Closure Rate	> 95%

SECTION 9: REGULATORY CONTACTS

Quality Department:
quality@hospital.org

Compliance Office:
compliance@hospital.org

Privacy Officer:
privacy@hospital.org

Medical Superintendent:
ms@hospital.org

Hospital Administrator:
admin@hospital.org

Emergency Compliance Hotline:
1800-000-0000

SECTION 10: RAG QA EXAMPLES

Question:
What records require informed consent?

Answer:
Surgery, invasive procedures, anaesthesia, blood transfusion, high-risk treatment, telemedicine, and research participation.

Question:

Who should receive a data breach report?

Answer:

Compliance Officer, Privacy Officer, Department Head, and Hospital Administrator.

Question:

How long should medical records be retained?

Answer:

Medical records should be retained for 10 years.

Question:

What is the escalation path for a serious compliance incident?

Answer:

Department Manager → Quality Manager → Medical Superintendent → Hospital Administrator → CEO.

Question:

What are the patient's core rights?

Answer:

Privacy, confidentiality, informed consent, dignity, safe care, information access, treatment refusal, and grievance resolution.